AI for Health Equity Design Sprint - Track 2

Hospital Financial Assistance Compliance Tracker

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In Brief

Build a self-updating tool that reviews hospital websites and financial assistance documents across to New York State and reports whether hospitals are compliant with the law. It presents the results in a public dashboard and map.

The Challenge

New York law requires all hospitals in the state to provide financial assistance to patients with incomes up to 400% of the federal poverty level (FLP). This law was most recently updated in October 2024, with changes to eligibility cutoffs, provided discounts, and transparency requirements.

A review conducted by CSSNY in September 2025 (a year after the reformed law was implemented) found that only 14 out of the state's 212 hospitals were fully compliant with the law. Most hospitals had outdated or missing information on their websites, which can mislead patients and hinder the provision of financial assistance.

A self-updating tool that tracks every hospital's compliance with the reformed hospital financial assistance law (HFAL) would give advocates, regulators, and lawmakers a resource to push for increased enforcement of this law at city and state levels.

AI for Health Equity

If this challenge remains unaddressed, patients may miss out on financial assistance that they are eligible for. This could lead to medical debt or forgoing care. Due to the income cutoffs of the HFAL, this problem primarily impacts low to middle income populations, inherently linking it to health equity. If the lack of implementation data about HFAL is not addressed, these populations will receive care with an unnecessary financial burden.

This challenge also has downstream effects. People with medical debt are more likely to experience subsequent housing instability and may have more difficulty finding employment or obtaining insurance if they have medical debt in collections. Patients with medical debt also experience poorer mental and physical health, and higher mortality rates.

A component of the updated HFAL is that information on financial assistance must be posted in multiple languages if a large enough portion of the hospital's service area has a primary language that is not English. If this part of the law is not followed, whole patient populations could miss out on care. This may compound inequities observed with medical debt in New York, where low-income communities and communities of color bear a disproportionate medical debt burden.

Desired AI Solution

CSSNY hopes an AI solution can help streamline and automate the monitoring of hospital compliance with the HFAL by reviewing hospital websites and documents and presenting the results in the form of a publicly available dashboard that patients, advocates, regulators, and lawmakers can use.

The ideal solution would have three components:

Component	Tasks & Function
Compliance monitoring	➔ Reviews each hospital's website and determines whether the hospital is compliant or not compliant with each measure as provided by the update HFAL. ➔ The result is displayed as a YES/NO, with a link to each hospital's financial assistance page and information on when that page was last updated. ➔ The result for language lists the languages a hospital has available for its materials and the result for primary service area lists the encompassed counties. ➔ Has a feed to display recent updates to hospitals' financial assistance materials
Hospital dashboard	➔ Assigns each hospital a 'grade' based on its compliance with each HFAL measure and a grading rubric. ➔ Displays the grade within a dashboard, alongside a note on which parts of the law a hospital does not meet. ➔ For each area of non-compliance, it provides a plain language explanation of the law, so that non-experts can understand how financial assistance resources within a hospital might be falling short. ➔ The audience for the dashboard are advocates, regulators, and lawmakers.
Statewide map	➔ Presents a dynamic map of New York State, showcase degrees of compliance in different colors. ➔ When hovering over a hospital dot, the map displays name, address, and a link to its financial assistance page. ➔ The audience for the dashboard are advocates, regulators, and lawmakers.

The tool must accurately extract information from hospital websites and documentation (usually in PDF format), and it must prevent hallucination when information cannot be found or is not available. When information is unavailable, it should be listed as N/A. If information is unclear to the system, it should flag it as such for manual verification.

Considerations

The solution should have high accuracy and reliability. It should not present information that wrongly leads a patient to assume they will be eligible for financial assistance when they are not, and then undergo a procedure they will have trouble affording. The reverse is equally serious. The tool should avoid wrongly

steering away patients from a hospital because the information wrongly led them to believe they were not eligible for financial assistance.

Datasets & Resources

- List of New York Hospitals: https://profiles.health.ny.gov/directory/hospitals
- Definition of 'hospital' under HFAL: https://newyork.public.law/laws/n.y._public_health_law_section_2807-k
- Grading rubric for HFAL compliance.
- Spreadsheet with results from 2025 manual review: https://docs.google.com/spreadsheets/d/17S2EheUVVr1g911fPpBQJqRpy18pkUlaEEnvGM0LB1o/edit?usp=sharing
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INSTRUCTIONS – *These were the instructions that were used for the manual creation of the database in Sept 2025. The instructions about pulling documents into SharePoint (among some others) are not relevant for the AI challenge, however I am including these instructions to give a sense of what we did to gather the information originally*

1. Look at the cell of the Hospital and check what System it is a part of. Use the filter of the "System" column to filter to all hospitals within that system.
    a. NOTE: Some systems still have **different materials** for each Hospital, but this is a good way to check this all in the same place. If you are 100% sure that all the Hospitals in the system have the same information, this is also an easy way to replicate/auto-populate that information for all hospitals in the system.
2. Download all posted application materials and add them to the SharePoint. Add them all as PDFs (if it is a Word document, download it and convert it, and then add it) (can add more details on this)
    a. If it is a website landing page or not downloadable, take a screenshot (jpeg). Make sure it is legible. Upload it.
3. Fully read through all the posted application materials. Take note of anything that falls in the pale-yellow columns and start filling them out.
    a. Start by noting what materials are posted, and then languages available.
    b. Fill out the other columns in an order that makes sense. Read carefully for time frame to submit, immigration status, asset checks, and primary service area, as information on these can be sprinkled throughout all posted documents and is not in a specific order.
4. Look at the posted application. For the "Application" section (blue) fill this out thinking ONLY about the application. Pretend the other documents do not exist.
    a. For example, the application has 2024 FPL guidelines but everything else has 2025 FPL guidelines. For this section, put down that the guidelines are 2024 FPL. Alternately, if every other document incorrectly caps FA at 300% FPL, but the application is the Uniform Application and properly places the threshold at 400% FPL, put that the cutoff is 400% FPL. This guards against double awarding or double penalties. HOWEVER, note any of these inconsistencies in the inconsistencies' column under the pale-yellow section, or the "Other notes" section, if more appropriate.
5. Look at the Plain Language Summary (/Policy Summary, these are used interchangeably). Fill out the "Plain Language Summary" section (purple) as if no other documents exist. ONLY look at the PLS—even if the policy has some of this information that is different
6. Go back to the inconsistencies column and note any more inconstancies. Note anything pertinent in "Other notes." I paste the definition of the PSA (directly quoted) in this section
7. Note any "judgement calls" or questions you have on the Methodology document. Nothing is too small to write down!!
8. ONLY IF YOU ARE 100% SURE!!!! Replicate the information for other hospitals in the system. Make sure to make a folder for EACH OF THEM on SharePoint, and add all appropriate PDFs
9. Turn off the filter on the "System" column.

COMPLIANCE GRADES

Compliant = compliant in all sections of the audit

Non-compliant = one or more sections of the audit are non-compliant

A	Fully compliant hospitals
B	1 to 3 errors (across all sections)
C	4 to 6 errors (across all sections)

D	7 to 9 errors (across all sections)
F	10+ errors OR one of the following errors is an automatic F: - No application posted OR no uniform application - No policy statement posted

COLUMN BY COLUMN REFERENCE (The 17 categories in the sheet that are binary are highlighted)

		Column title in Excel	Explanation of content & applicable notes
	M	System	The health/hospital system the hospital belongs to (e.g. NYC Health + Hospitals)
	N	HFA Link	The link to the part of the website related to the hospitals HFA (the format of what the link arrives at may vary, but should be able to access all available HFA components from this link)
	O	Compliance Grade	Graded based on chart above
MATERIALS ON WEBSITE	R	Application Posted	Is a financial aid application posted? **YES** = an application is posted (any application, it does not have to be the UA; this is just evaluating if ANY APPLICATION is posted) **NO** = there is no application on the website
	S	Policy Summary Posted (as a PDF, or on landing page)	Is the policy summary posted and accessible? **YES** = it is **NO** = it is not/it is missing
	T	Policy Statement posted	Is the policy statement posted and accessible? **YES** = it is **NO** = it is not/missing
	U	Consistent across posted documents (Y/N)	Is the information in all the posted materials consistent? **YES** = it is **NO** = it is not—refer to notes in next column to find out why (this will include if there is a non-compliant online application, this will be counted as INCONSISTENT. Additionally, if there are supplemental documents with inconsistent information, this is also inconsistent.)
	V	Primary Service Area boundaries compliant with 2007 guidelines?	This column refers to if the definition of the PSA is compliant with 2007 law, REGARDLESS of where it is listed. **YES** = the PSA is compliant wherever it is listed **NO** = it is not compliant (e.g. the wrong areas are listed). If a summary lists a PSA but it is INCORRECTLTY DEFINED this column will say NO
	W	No time limit to submit?	By law, there should be NO time limit to submit. **YES** = the hospital is compliant with this, they do not have a submission time limit **NO** = there is a time limit listed
	X	Notes on timeframe	This section is for notes on the timeframe, if there is one (e.g. what is the length, is it proactive or retroactive)
	Y	No evidence of asset checks?	**YES** = compliant, asset checks are illegal **NO** = evidence can be a direct asset check, asking bank account statements

	Z	No mention of immigration status as a barrier	**YES** = Compliant, no mention of exclusion based on immigration status OR an explicit statement that immigration status does not matter **NO** = Non-compliant, mentions exclusion from financial assistance based on immigration status
	AA	Inconsistency notes	THIS COLUMN IS USED FOR INCONSISTENCIES BETWEEN THE DOCUMENTS INTERNAL TO THE HOSPITAL - This is for things that differ between the application, summary, and policy This section does semi-relate to the law (e.g. if something is off in all the documents, it will be noted here as well), but it is also about things being inconsistent from document to document
	AB	Languages available	Lists the languages the application is available in - By law, hospitals must have applications in the "primary languages" of patients served by the general hospital *"primary languages" shall include any language that is either (i) used to communicate, during at least five percent of patient visits in a year, by patients who cannot speak, read, write or understand the English language at the level of proficiency necessary for effective communication with health care providers, or (ii) spoken by non-English speaking individuals comprising more than one percent of the primary hospital service area population, as calculated using demographic information available from the United States Bureau of the Census, supplemented by data from school systems.*
	AC	Notes (e.g. use of "charity care")	Notes on any of the application documents posted
APPLICATION	AD	ENTIRE Uniform Application (Y/N)	Is the hospital using the NYS uniform application? - By law, all hospitals need to use the uniform form: *"...utilizing only a uniform financial assistance form developed and provided by the department. All general hospitals that do not participate in the indigent care pool shall also utilize only the uniform financial assistance form..."* **YES** = they are using the FULL UA **NO** = the hospital is using a different application, or only part of the UA. IF THEY ARE ONLY USING PART OF THE UNIFORM APP, this section will be marked NO
	AE	Contact information included in application	**YES** = they have contact information clearly listed on APPLICATION **NO** = there is no information clearly listed
	AF	Contact phone #	The primary contact phone # listed for HFA help
	AG	Income & discount tables included in application	**YES** = the application directly includes tables that show the income levels for HFAL and corresponding discounts on care **NO** = this is not included in the application
	AH	2025 FPL Guidelines	Is the application using 2025 FPL guidelines? **YES** = it has been updated to 2025 guidelines **NO** = it is based off another year (or could be missing this entirely)

	AI	Max FPL (compliant is =/>400%)	This column states the maximum % of the FPL that the specified hospital's HFA covers. By law, HFA should apply to those at or below 400% of the FPL, so compliance in this column is indicated by a number that is **400% or higher**. **YES** = the number in this column is >=400% **NO** = the number is <400%
	AJ	Installments capped at 5% of monthly income	For payment plans/installments, is it explicitly stated that the monthly payments are capped at 5% of gross monthly income (as required by law) **YES** = this is explicitly stated in the application **NO** = it is not explicitly stated anywhere
	AK	Application notes (any prohibited information?)	USED ONLY FOR NOTES ABOUT THE APPLICATION - Notes here relate to something being asked by the application that is prohibited - Notes can also just be about the application in general, e.g. if there is incongruencies about FPL between the app and the law they go in this column - Notes here can also explain if a hospital is using the uniform application but OMITTED part of it (e.g. the installments clause)
POLICY SUMMARY	AL	Primary Service Area listed in summary?	As required by law, the PSA must be listed in the hospital's summary. **YES** = the summary describes the PSA **NO** = the summary does not list the PSA (this section ONLY refers to the summary—if the PSA is listed in the policy or elsewhere this column will still say NO) LIST of primary service areas from each county, from DOH: Patient Financial Assistance Letter
	AM	Means of applying for assistance explained	Does the policy summary explain how to apply for assistance? **YES** = it does **NO** = it does not
	AN	Income levels used to determine assistance	Does the policy summary include information on income levels that will determine assistance ($ amounts, % of FPL, etc.)? **YES** = it does **NO** = it does not
	AO	Notes	Any notes about the Policy Summary

Compliance by category from September 2025 project

Percentage (number) Compliant	Category
98% (208)	have an **application** posted
98% (207)	do not limit financial assistance due to immigration status
88% (186)	have a **policy summary** posted
88% (186)	have an explanation of how to apply for financial assistance in the **policy summary**
86% (182)	state in the **application** that that the income limit for financial assistance is 400% of the FPL (or higher)
85% (180)	state in the **application** that installments are capped at 5% of monthly income
83% (177)	have contact information included in the **application**
82% (174)	have income and discount tables included in the **application**
82% (174)	have income levels used to determine assistance in the **policy summary**
81% (171)	use the ENTIRE **Uniform Application**
76% (162)	do not collect information that could be used to conduct illegal asset checks
74% (157)	have no time limit to submit
72% (153)	have primary service area boundaries compliant with 2007 guidelines
55% (117)	have the primary service area described in the **policy summary**
50% (106)	use 2025 FPL guidelines in the **application**
49% (104)	are consistent across posted documents (e.g., always use the same year of FPL, always use the same discount tables)